



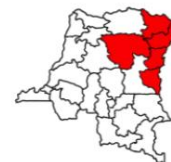
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°111/MVEBDB/02/09/2026

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HIGHLIGHTS (last 24 hours)

In the last 24 hours, 92 new confirmed cases of Ebola Virus Disease have been reported, including 24 community deaths. (EVD) were recorded in the provinces of Ituri (60 cases), North Kivu (25 cases), Haut Uélé (5) and (2) in Tshopo.

In addition, 9 intra-CTE deaths were recorded.

No new health zones affected in the last 24 hours, with overall notification increasing (+53.1%)

ACCUMULATIVE CASE 6,342	ACCUMULATIVE DEATH CONFIRMED 3,072	LETHALITY 48.4%	PATIENTS IN ISOLATION/CTE 770	ACCUMULATIVE HEALED 1,475	TRACKING RATES CONTACTS (Of the Day) 86.5%
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo & Bas Uélé



60 out of 151 health zones affected (39.7%)



Summary of key points from the last 24 hours

Over the past 24 hours, **92 new confirmed cases** of Ebola Virus Disease (EVD), including **24 community deaths**, have been reported in the provinces of **Ituri (60 cases)**, **North Kivu (25)**, **Haut-Uélé (5)**, and **Tshopo (2)**. In addition, **9 deaths in Ebola Treatment Centers (ETCs)** were recorded: 6 in Ituri, 2 in North Kivu, and 1 in Haut-Uélé, bringing the **total number of deaths for the day to 33**.

However, **36 patients** were declared cured after obtaining two negative results for Ebola virus disease (28 in Ituri, 6 in North Kivu and 2 in Haut Uélé).

As of September 2, 2026, the DRC has a total of **6,342 confirmed cases, including 3,072 deaths**, representing a case fatality rate of **48.4 %**. Ituri province remains the epicenter with **81.4%** of confirmed cases recorded in the country and holds **65.2%** of new confirmed cases reported in the last 24 hours.

Since the start of the epidemic in the DRC, **60 out of 151 health zones (39.7%)** remain affected across the six affected provinces. Ituri has 28 affected health zones out of 36, followed by North Kivu (15/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas Uélé (3/11).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 2, 2026.

Province	New cases confirmed (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	60	5164	2,342	45.4%	28/36 (77.8%)
North Kivu	25	913	614	67.3%	15/34 (44.1%)
Haut-Uélé	5	236	103	43.6%	6/13 (46.1%)
Tshopo	2	22	9	40.9%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas Uélé	0	4	3	75.0%	3/11 (27.3%)
Total	92	6,342	3,072	48.4%	60/151 (39.7%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 2, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5164	2342	45.4%	60	12	6	18
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	168	36	21.4%	3	0		0
Boga	2	2	100.0%				0
Bunia	1444	447	31.0%	31	5		5
Damascus	28	9	32.1%				0
Drodro	14	7	50.0%				0
Fataki	77	30	39.0%				0
Gethy	6	2	33.3%				0
Kambala	2	0	0.0%				0
Kilo	32	12	37.5%				0
Komanda	92	62	67.4%				0
Lita	225	121	53.8%	1	0		0
Logo	11	5	45.5%				0
Lolwa	22	5	22.7%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	36	21	58.3%				0
Mangala	271	138	50.9%	8	4		4
Mongbwalu	616	288	46.8%	1	1		1
Nia-Nia	218	113	51.8%	2	0		0
Nizi	675	266	39.4%	5	2		2
Nyankunde	124	35	28.2%				0
Rimba	11	4	36.4%				0
Rwampara	971	341	35.1%	7	0		0

Tchomia	59	25	42.4%	2	0		0
To ventilate	N / A	351	N / A	N / A	N / A	6	6
North Kivu	913	614	67.3%	25	11	2	13
Blessed	151	109	72.2%	5	3	1	4
Good	1	1	100.0%				0
Butembo	186	134	72.0%	6	0		0
Goma	1	0	0.0%				0
Kalunguta	16	7	43.8%				0
Katwa	416	278	66.8%	8	7	1	8
Kyondo	16	12	75.0%				0
Lubero	4	3	75.0%	1	0		0
Mabalako	7	5	71.4%	1	0		0
Manguredjipa	2	1	50.0%				0
Masereka	12	5	41.7%	1	0		0
Musienene	85	48	56.5%	2	1		1
Mutwanga	2	2	100.0%				0
Oicha	8	6	75.0%	1	0		0
Vuhovi	6	3	50.0%				0
Haut-Uélé	236	103	43.6%	5	1	1	2
Boma	27	14	51.9%				0
Mangbetu							
Gombari	3	1	33.3%				0
Isiro	86	35	40.7%	1	1		1
Pawa	39	22	56.4%	2	0		0
Rungu	2	1	50.0%	1	0	1	1
Wamba	79	30	38.0%	1	0		0
Tshopo	22	9	40.9%	2	0	0	0
Bafwasende	2	0	0.0%				0
Kabondo	4	1	25.0%	1	0		0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%	1	0		0
Mangobo	3	2	66.7%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	4	3	75.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				0
Ganga	2	2	100.0%				0
Total	6342	3072	48.4%	92	24	9	33

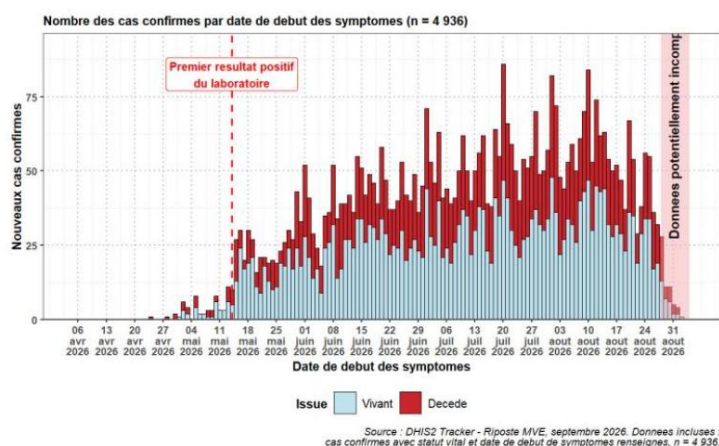
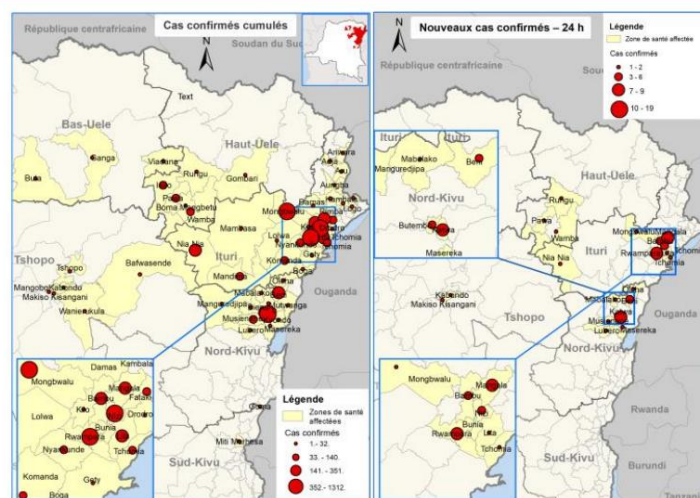
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

** These deaths were recorded in the various CTEs of the Ituri province and are currently being assessed.

Table 3. Status of alerts notified by province, as of September 2, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	24	6	30	21	6	3	0	27	15
Bas Uélé	33	1	34	5	1	28	0	6	0
Ituri	900	66	966	140	66	746	0	206	119
North Kivu	1,374	74	1448	123	57	1251	17	180	100
South Kivu	13	0	13	5	0	1	0	5	4
Tshopo	118	1	119	9	0	106	0	9	2
Total General	2462	148	2,610	303	130	2135	17	433	240

Figure 1. Evolution of cases by date of symptom onset over the last 21 days**Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.**

1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

During the course of September 2, 2026, 2,610 alerts were recorded in the 6 affected provinces, of which 2,585 (99.0%) were verified. Following this exercise, 433 alerts were validated as suspected cases. These were all investigated (100%), and 240 of them were transferred to the CT/CTE (Table 3).

Regarding contacts monitored in the last 24 hours, the proportion is 86.5% (17,910 seen out of 20,711 to be monitored), less than the previous day (89.0%) but still above the 85% threshold; 59.8% in Tshopo (211/353), 84.9% in North Kivu (7,674/9 034), 89.8% in Ituri (9,310/10,371), 80.0% in Haut Uélé (471/589) and 67.0% in Bas Uélé (244/364). Data from the Wamba health zone (Tshopo province) were not reported.

1.1.3. Challenges

Persistent low contact tracing around confirmed cases in the Lower Province Uélé, linked to a lack of teams dedicated to these activities.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

Nine (9) live alerts were notified to the various PoE/PoC as of September 2, 2026, two of which were validated: one in Ituri, at the Dele PoC (Rwampara Health Zone), which was isolated but refused transfer to the CTE; and one in Haut-Uélé, at the Magambe PoC (Isiro Health Zone), which was intercepted, validated, and referred to the CTE. The three alerts from North Kivu and the four from Bas-Uélé were invalidated.

Table 4. Status of key indicators for PoC/PoE by province, as of September 2, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Bas-Uélé	Overall
Report completeness	85% (51/60)	100% (18/18)	55.5% (5/9) 83%	15/18) 64% (7/11)		ND	82.8% (96/116)
% of functional strategic PoE/PoC	9.1% (1/11)	N / A	N / A	N / A	N / A	N / A	9.1% (1/11)
Number of people who have switched to PoE/PoC	128,104	87,136	8,848	13,299	9,890	5,250	252,527
% of travelers screened	97.49%	97.66%	98.87%	100%	98.86%	98.00%	97.80%
% of travelers who washed their hands	98.08%	97.66%	98.87%	98.56%	95.17%	ND	97.87%
% of travelers made aware	99.62%	97.66%	100%	100%	95.56%	ND	98.80%
Number of alerts notified	1	3	0	0	1	4	9
Number of validated live alerts (suspected cases)	1	0	0	0	1	0	2
Number of bodies intercepted today; % of suspected	0	0	0	0	0	0	0
cases transferred to CT/CTE	0% (0/1)	N / A	N / A	N / A	100% (1/1)	N / A	50% (1/2)
% of lifeless bodies swabbed/ secured	N / A	N / A	N / A	N / A	N / A	N / A	N / A
% of suspected cases investigated within 24 hours	100% (1/1)	0	N / A	N / A	100% (1/1)	0	(2/2) 100%

1.2.2. Challenges

Operationalization of PoE/PoC facilities is suboptimal, and reporting is low in South Kivu (55.5%) and Haut-Uélé (64.0%). In Ituri, only 1 (9.09%) of the 11 strategic PoE/PoC facilities remains fully operational: the Avakubi PoC is non-functional for security reasons, while the Nzebi, Frigo, Galay, Pont Ituri, Makoko 2, Mabakese, Bey, and Foner Mambasa PoC facilities are experiencing strikes due to non-payment. In Haut-Uélé, 4 PoC facilities (Dusu, Apodo, PK6 Niangara Road, and Gossamu) have not reported, and the Magambe PoC is experiencing high rates of handwashing refusal (12%). In South Kivu, the lack of isolation facilities at the Kavimvira, Kalundu Port SNCC, and Baraka PoE facilities persists. In Bas-Uélé, insufficient mobility, connectivity, and water supply to PoE/PoC persists.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 60 new positive results** (48 living and 12 dead) out of 267 new samples received and analyzed (**positivity: 22.5 %**).
- **North Kivu : 25 new positive results** (14 living and 11 dead) out of 110 samples analyzed (**positivity of 22.7%**).

- **Haut-Uélé: 5 new positive results** (4 living, including 1 death) out of 21 samples analyzed (**positivity: 23.8%**).
- **Bas-Uélé: 4 samples** received and tested (4 blood samples), all came back negative.
- **Tshopo: 2 new positive results** (2 live) out of 6 samples analyzed (**positivity: 33.3%**).

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 18 rings were opened out of the 55 identified (32.7%), 133 households decontaminated out of 143 identified (93.0%) and 11 FOSA out of 11 (100%); 72 EDS carried out out of 91 alerts and 56 bodies swabbed.
- **In North Kivu**, 19 out of 38 rings were opened (50.0%), 14 out of 35 identified households were decontaminated (40.0%), and 34 out of 39 health facilities were decontaminated (87%); 6 health facility assessments were carried out with an average score of 29%. In addition, 64 bodies were swabbed and 59 blood and sanitation assessments (BSAs) were conducted.
- **In Tshopo**, continued decontamination of 6 premises, 1 latrine and 1 shower at PEKIS (17th case); monitoring and support of 11 ESS in 5 ZS (Makiso-Kisangani, Mangobo, Tshopo, Kabondo and Lubunga) during which 116 providers were briefed on the implementation of IPC measures; decontamination of 6 premises, 1 latrine and 1 shower at PEKIS and implementation of a screening and triage system at the Bafwasende General Referral Hospital.

1.4.2. In Bas-Uélé, evaluation of the HGR of the ZS of Ganga (score: 21.3%) and 1 swabé body in the ZS of Ganga.

1.4.3. Challenges

- Weak implementation of IPC standards: In Ituri, the opening of infection control rings remains low (32.7%, 18/55); insufficient decontamination teams in Fataki, Mangala, and Nizi. In North Kivu, only 14 households were decontaminated out of 35 identified (40.0%). In Bas-Uélé, no rings were opened and no households decontaminated out of the 2 identified, with 5 health zones remaining without IPC training (Aketi, Bondo, Likati, Monga, and Bili). In Tshopo, there is a shortage of IPC supplies in health centers and the incinerator at Makiso-Kisangani General Referral Hospital is out of service.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, there were 63 new admissions to the various dedicated facilities, compared to 106 discharges, including 28 recoveries. Thus, 434 patients are hospitalized in 978 beds, representing an occupancy rate of 44.4%.
- **In North Kivu**, 73 new admissions and 50 discharges were recorded, including 6 recoveries; 261 patients are hospitalized and the overall occupancy rate of beds in CTE/CT is oversaturated (118.6%; 220 beds available).
- **In Haut-Uélé**, 9 new admissions were recorded. By the end of the day, 56 patients, representing an overall occupancy rate of 46.7% (120 beds).
- **In Tshopo**, 3 new admissions were recorded. Seven (7) patients are in isolation for 25 beds, representing an overall occupancy rate of 28.0%.

1.5.2. Challenges

Low capacity and saturation: in Ituri, the Nizi CTE is saturated for suspected cases; in North Kivu, CT/CTEs are oversaturated (118.6%), some patients are treated in non-dedicated ESSs; in Bas-Uélé, there is no standard CTE/CT and no ambulance; in Tshopo, there is no standard CTE in Bafwasende and no blood bags are available.

1.6. Risk communication and community engagement

1.6.1. Main actions

- **In Ituri**, CREC support to the EDS around 52 community deaths in 10 health zones of Ituri including Nizi, Mongbwalu, Bambu, Fataki, Mangala (Djugu Territory),

Bunia, Komanda, Rwampara (Irumu Territory) Nia-Nia (Mambasa Territory), Tchomia (straddling Djugu and Irumu). Aru (Aru Territory); Advocacy with the district president CE39 Songolo (Gety Health Zone); Chief of the Plutô Group (Mongbwalu Health Zones).

- **In North Kivu**, 6,033 home visits were carried out, reaching 19,680 people; 83 educational talks were conducted with 3,142 people; 1 briefing (100 people), 1 advocacy and 2 community dialogues (102 people); 11 feedback and infodemics were resolved within 48 hours out of 15 captured and 281 alerts were notified by community actors.
- **In Tshopo**, 3,634 people sensitized (1,978 women and 1,656 men); 2 rumors clarified and 2 community feedbacks collected; 514 leaflets, 78 posters and 4 banners distributed; support for the launch of local polio vaccination days at the Pont Tshopo roundabout, with 2,720 people sensitized; training of community leaders of AS Lundi and Kenge (ZS Bafwasende).
- **In Bas-Uélé**, 1,278 people were sensitized in health facilities and among pregnant women; briefing of 50 representatives of the motorcyclists of Buta; round of evaluation of hand washing in schools in Buta; demonstration of chlorine preparation in schools in the ZS of Bili; support for community-based surveillance in the ZS of Monga and interactive broadcast on Rubi radio in Buta.

1.6.2. Challenges

Persistence of challenges at community level: in Ituri, 21 refusals of isolation at the CTE, 11 bodies not swabé due to community resistance and resistance in Rwampara, Nyankunde, Bunia, Tchomia, Lita, Mangala and Kilo; in Tshopo, resistance to investigations, sampling, isolation and transfer to the CTE.

1.7. Mental Health and Psychosocial Support (MHPSS)

1.7.1. Main actions

- **In Ituri**, 16 people among the new confirmed cases and 143 of the 176 confirmed cases under follow-up (81.2%) benefited from SMSPS interventions; 27 of the 63 new suspected cases (42.8%); 68 households received support, 60 people with HIV received assistance, and 64 test results were announced, including 16 positive cases and 16 recovered out of 17 monitored in the community (94%).
- **In North Kivu**, 18 new confirmed cases and 89 of the 108 suspected cases benefited from SMSPS interventions; 103 of the 123 test results were announced, including 20 positive cases, 68 people receiving support and 40 households receiving assistance. In Tshopo, 2 cases were confirmed and 2 suspected cases are under follow-up; 3 test results were announced, including 2 positive cases, and 6 caregivers received support at the treatment center; 2 recoveries have joined the response teams.

1.7.2. Challenges

In Ituri, low coverage of SMSPS interventions around the latest confirmed cases; coverage limited to 5 of the 9 health zones that notified (55.6%).

1.8. Logistics

1.8.1. Main actions

- **In Ituri**, work began on leveling the site of the Mangala Ebola Treatment Center (ETC) and inter-ETC transfers were organized. • **In North Kivu**, a joint visit was conducted to the ETCs of Katwa, Beni, and Matanda, and the land intended for the future Musienene treatment center was surveyed, with medical PEC, WHO, WFP, and UNICEF.
- **In Bas-Uélé**, support for the deployment of the surveillance team for contact tracing, alert verification and active research, and continuation of vaccination of PPL and TPL in the Buta health zone.

1.8.2. Security

1.8.3. Main actions

- In North Kivu, 9 activities to maintain public discipline were carried out, including security support for IPC teams for decontamination in Beni (AS Malepe, Butsili and Mukulya) and EDS teams for the burials of 4 confirmed cases in the cemeteries of Mankizo and Bundji.

1.8.4. Challenges

Twenty-one (21) refusals to self-isolate at the CTE were reported in Ituri. In North Kivu, officials resisted compliance with the health protocol at the PoCs, and security was reinforced at the Vulindi PoC (Butembo Health Zone).

1.9. PSEA (Prevention of Sexual Exploitation and Abuse)

1.9.1. Main actions.

In Ituri, training continues on the prevention of sexual exploitation, abuse and harassment in the various affected provinces.

1.9.2. Challenges

- Still insufficient coverage of PSEA activities: 25% of the health zones in Ituri (9/36) are aware of the reporting mechanism; lack of mapping of stakeholders; insufficient mass awareness materials; lack of means of transport and computer equipment for the pillar.

KEY MESSAGES

- Continued transmission of EVD, with **92 new confirmed cases** including **24 community deaths and 9 intra-CTE deaths** recorded in the last 24 hours, bringing the national total to **6,342 confirmed cases and 3,072 deaths**.
- Performance levels are maintained in epidemiological investigations (**100% of suspected cases investigated**) and the daily proportion of contact tracing remains stable (**86.5%**) which remains above the threshold of 85.0%.
- Ituri remains the epicenter of the epidemic, concentrating 81.4% of cumulative confirmed cases and 65.2% of new cases reported in the last 24 hours, followed by North Kivu province which has the highest lethality.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic is progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a national and then continental public health emergency, accompanied by mobilization funds.



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